next2u SAÚDE

Early-warning home monitoring for chronic patients in Brazil's public primary care

A wristband, an Android app and a 158-rule clinical engine that tell the care team who is getting worse, before it becomes a hospitalization.

Seed round · 2026 · next2u.com.br

PROBLEM

Chronic patients deteriorate at home, between visits, where primary care can't see them

29.7%

of adults in Brazilian state capitals report a hypertension diagnosis (2024).

12.9%

of adults report diabetes, more than double the 5.5% of 2006.

738k

deaths from chronic non-communicable diseases in 2019, 54.7% of all deaths in Brazil.

Follow-up is episodic

Community health teams see a patient every few weeks. Blood pressure, oxygen saturation and heart rate in between are unknown.

Decompensation is found late

Many hospitalizations of hypertensive, diabetic, kidney, lung and liver patients are classed as sensitive to primary care: avoidable if caught early.

Sources: Ministério da Saúde, Vigitel 2006–2024 series (capitals + Federal District); Plano de Ações Estratégicas para o Enfrentamento das DCNT 2021–2030 (2019 data). Primary-care-sensitive conditions: Portaria MS nº 221/2008.

SOLUTION

Continuous vital signs, rule-based alerts, one panel per health unit

Patient

Wear and forget

A low-cost BLE wristband measures heart rate, SpO2, blood pressure, temperature and sleep. The Android app uploads readings and queues them offline when there is no signal.

Engine

158 clinical rules

Each reading is checked against 158 base alerts, expanded to 971 patterns by hospitalization risk, active diseases and medication. A score or a drug alone never fires an alert.

Care team

Who needs attention now

The UBS panel ranks patients by alert priority, inside the team's own territory. Every alert is decision support: the clinician decides the conduct.

Outcome we are built for: fewer avoidable hospitalizations and earlier home visits for the patients who need them.

HOW IT WORKS

One critical path, tested on every commit

01
Wristband
HBand / Veepoo SDK or any watch with the standard Bluetooth health profile.

02
Companion app
Kotlin Android app, pairing, offline outbox, retries.

03
Secure ingest API
Scoped API keys, rate limiting, per-patient authorization, idempotent writes.

04
Alert matrix
158 rules → 971 patterns, weighted by risk, disease and medication.

05
UBS panel
Territory-scoped view; separates HTTP, simulated and physical-device data.

Interoperable
HL7 FHIR R4 export (Patient, Observation, Device, Flag, Bundle).

Cloud-native
Bronze / Silver / Gold lakehouse on Google Cloud: Cloud Run, BigQuery, Cloud Storage.

Private by design
Hashed identifiers, territory-based access, LGPD impact report and incident plan.

PRODUCT

Clinical protocols for the conditions primary care already follows

PRIORITY	BASE ALERTS	BRANCHES	PATTERNS
★ Mild	43	7	301
★★ Moderate	70	7	490
★★★ Severe	45	4	180
Total	**158**	—	**971**

Mild and moderate alerts are promoted only with confirmation, persistence, a second parameter or an additional clinical sign.

Six care pathways

- Hypertension
- Diabetes
- Chronic kidney disease
- Respiratory disease (COPD, asthma)
- Liver disease
- High-risk pregnancy

Hospitalization risk score

Multimorbidity, heart or liver failure, cancer, polypharmacy, recent admission, cognitive impairment and social isolation set the patient's baseline tier, from standard follow-up to intensive home care.

MARKET AND CUSTOMER

The buyer is the municipal health secretariat; the unit is the UBS

5,570

municipalities in Brazil, each responsible for primary care in its territory (IBGE).

1 in 3

adults in the capitals lives with hypertension or diabetes, the core of the UBS chronic-care caseload.

Bottom-up sizing

Monitored patients per municipality × price per patient per month. Our first segment: high-risk tiers (score ≥ 11) that need telemetry.

[Fill: price/patient/month and target TAM]

Why municipalities

Primary care is organized by territory and health unit, which maps directly to our access model and alert routing. Avoidable admissions are a cost the municipality and the state pay.

Expansion

The same engine and API serve health plans and hospital-at-home programs once the public-sector pilot produces outcome data.

"1 in 3" is a conservative reading of Vigitel 2024 (hypertension 29.7%, diabetes 12.9%, with overlap). Municipality count: IBGE.

BUSINESS MODEL

B2G SaaS: per monitored patient, per month

Recurring

Platform subscription

Per active monitored patient per month: app, ingest, alert engine, panel, support.

[Fill: price]

One-off

Device kit

Wristband supplied at cost or through the municipality's own purchase. We stay device-agnostic.

Services

Implementation

Protocol set-up per pathway, team training and integration with the municipal health record via FHIR.

Go-to-market

Paid pilot with one municipality → outcome report → public procurement or innovation partnership → neighboring municipalities and regional health consortia.

Unit economics

Serverless API on Cloud Run with per-request billing; the rule engine runs on CPU. Infrastructure cost per patient is tracked in our GCP billing ledger.

[Fill: cost/patient/month and target margin]

WHERE WE ARE

Product built and running end to end; physical-device pilot next

Done

- **Android app v1.5**, installable, with offline outbox
- **Secure ingest API** with scoped keys, rate limit and per-patient lock
- **158-rule alert engine** with decision-support output
- **UBS panel** with territory-scoped access
- **HL7 FHIR R4** export and Medallion lakehouse
- **Automated CI**: critical-path tests on every commit, Linux and Windows, PostgreSQL
- **LGPD file**: impact report, processing record, retention, incident plan

Next 6 months

- Integrate the HBand / Veepoo SDK on physical wristbands
- First municipal pilot with real patients and real devices
- Measure alert precision and time-to-visit against the team's routine
- Regulatory assessment under ANVISA RDC 657/2022 (software as a medical device)

Stage: pre-revenue. Simulated-device sessions are not counted as pilot data.

AI ROADMAP AND EVIDENCE DISCIPLINE

Rules in production today; prediction only after it is proven

Research track: deterioration forecasting

A temporal convolutional network (PyTorch) forecasts deterioration at 6, 24 and 72 hours. We keep it out of the product because training and real-world results diverge:

F1 on training data · 0.98



F1 on real labels (5 patients, 752 sequences) · 0.64



We will expose a predictive horizon only after patient-level validation on people outside the training set.

Where NVIDIA accelerates us

- **CUDA** for training temporal models on pilot telemetry as it grows
- **RAPIDS cuDF** for lakehouse feature pipelines
- **TensorRT + Triton** for low-latency inference once a model passes validation

The alert matrix stays the source of truth for the pilot. AI adds value when it beats the rules on real patients, not before.

POSITIONING

Built for public primary care, not adapted to it

	CONSUMER WEARABLE APPS	TELEHEALTH PLATFORMS	HOSPITAL RPM SUITES	NEXT2U SAÚDE
Continuous home vital signs	●	○	●	●
Clinical rules by chronic pathway	○	◐	●	●
Territory / UBS access model	○	○	○	●
Device-agnostic, low-cost hardware	○	◐	○	●
FHIR R4 interoperability	○	◐	●	●
Priced for municipal budgets	◐	◐	○	●

● yes · ◐ partial · ○ no. Category-level comparison; individual vendors vary.

SAFETY, PRIVACY, REGULATION

Decision support with guardrails

Clinical safety

Every alert is named "Possible..." and carries decision support, never a mandatory conduct. A risk score, disease or drug alone cannot trigger an alert.

LGPD

Data protection impact report, processing record, retention policy and incident response plan. Identifiers are hashed; access is limited to the patient's territory.

Security

Scoped API keys, rate limiting, anti-IDOR checks per patient, idempotent ingest, dependency audits in CI.

Regulatory path: classification under ANVISA RDC 657/2022 for software as a medical device is part of the pilot workstream. We do not claim certification today.

TEAM

The people building it

[Name]

[Role]

[One line of background]

[Name]

[Role]

[One line of background]

[Name]

[Role]

[One line of background]

[Name]

[Role]

[One line of background]

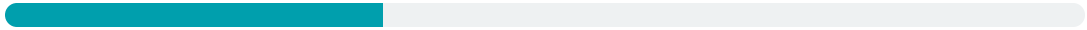
Clinical advisors: [Fill: names and specialties]

THE ASK

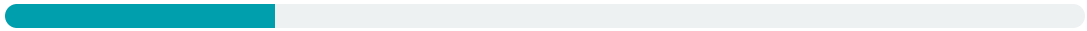
Seed round of [US$ amount] to prove outcomes in the field

Use of funds

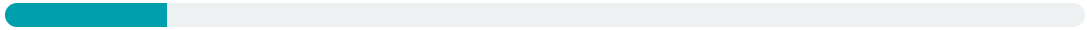
Field pilots: devices, deployment, municipal support [%]



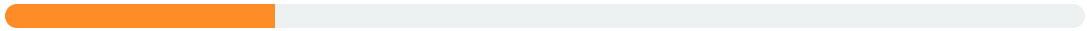
Clinical validation: prospective study, patient-level evaluation [%]



Regulatory and privacy: ANVISA, LGPD, security audit [%]



Team: engineering, clinical lead, sales to the public sector [%]



Bar lengths are illustrative until the allocation is set.

Milestones this round buys

- Physical wristbands live with real patients in the first municipality
- Outcome report: alert precision, time to home visit, admissions
- First recurring contract and a second municipality
- ANVISA classification defined
- Temporal model validated per patient, or retired

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See who is getting worse, before the emergency room does

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